

## Dr. Amandeep Singh vs Gurdial Singh & Ors on 25 May, 2022

ADDITIONAL BENCH

STATE CONSUMER DISPUTES REDRESSAL COMMISSION,  
PUNJAB, CHANDIGARH.

- 1) First Appeal No.45 of 2019  
Date of institution : 29.01.2019  
Reserved on : 09.05.2022  
Date of decision : 25.05.2022

Dr. Amandeep Singh, Associate Professor, Unit IV, Department of  
Medicines, Punjab Institute of Medical Sciences, Garha Road,  
Jalandhar.

.....Appellant/Opposite party No.2  
Versus

1. Gurdial Singh, aged 62 years, son of Sh. Lachmann Singh,  
resident of 309/14, Sawan Singh Colony, Near Markfed,  
Kapurthala.

.....Respondent no.1/Complainant

2. Punjab Institute of Medical Sciences, Garha Road, Jalandhar  
through its Managing Director.  
3. Smt. Sarabjit Kaur, Senior Staff Nurse, Punjab Institute of  
Medical Sciences, Garha Road, Jalandhar.  
4. PIMS Medical and Education Charitable Society, Garha Road,  
Jalandhar through its Chairman/President.  
5. Oriental Insurance Co. Ltd. G.T. Road, Opposite Narinder  
Cinema, Jalandhar through its Branch Manager.

.....Respondents /Opposite Parties

Present:-

|                         |                                                                |
|-------------------------|----------------------------------------------------------------|
| For the appellant       | : Sh. Hardeep Singh, Adv. with<br>Dr.Amandeep Singh, In person |
| For respondent no.1     | : Sh. D.R.Punia, Adv. with<br>Sh.Gurdial Singh, In person      |
| For respondent no.2 & 4 | : Sh. I. S.Bhatia, Advocate                                    |
| For respondent No.3     | : Ex-parte                                                     |
| For respondent No.5     | : Sh. Krishna Kant, Adv. for<br>Sh. J.P. Nahar, Advocate       |

AND

2

FA No. 45 of 2019

2) First Appeal No.54 of 2019

Date of institution : 07.02.2019  
Reserved on : 09.05.2022  
Date of decision : 25.05.2022

1. Punjab Institute of Medical Sciences, Garha Road, Jalandhar through Sh. Amit Singh son of Sh. Harjit Singh, its Resident Director/Authorized Signatory.

2. PIMS Medical and Education Charitable Society (Regd.), having its Administrative Office at Punjab Institute of Medical Sciences, Garha Road, Jalandhar through Sh. Amit Singh son of Sh. Harjit Singh, its Resident Director/Authorized Signatory.

....Appellants/OPs No.1 & 4

Versus

1. Sh. Gurdial Singh son of Sh. Lachmann Singh, resident of 309/14, Sawan Singh Colony, Near Markfed, Kapurthala.

.....Respondent No.1/Complainant

2. Dr. Amandeep Singh, Professor, Department of Medicines, Kalpana Chawla Govt. Medical College & Hospital, Karnal (Haryana)

3. Smt. Sarabjit Kaur, Senior Staff Nurse, Punjab Institute of Medical Sciences Garha Road, Jalandhar.

4. Oriental Insurance Company Ltd., G.T.Road, Opp. Narinder Cinema, Jalandhar through its Branch Manager.

.....Respondents/Opposite Parties

First Appeals against the similar orders dated 07.01.2019 passed by District Consumer Disputes Redressal Forum (now Commission) Jalandhar.

Quorum:-

Mr. H.P.S.Mahal, Presiding Judicial Member

Mrs. Kiran Sibal, Member Present:-

|                     |                                                            |
|---------------------|------------------------------------------------------------|
| For the appellants  | : Sh. I.S.Bhatia, Advocate                                 |
| For respondent No.1 | : Sh. D.R.Punia, Adv. with<br>Sh. Gurdial Singh, in person |
| For respondent No.2 | : Dr.Amandeep Singh,in person                              |
| For respondent No.3 | : Ex-parte                                                 |
| For respondent No.4 | : Sh. Krishna Kant, Adv. for<br>Sh.J.P. Nahar, Advocate    |

KIRAN SIBAL, MEMBER

This order shall dispose of the above noted two First Appeals preferred against the same order dated 07.01.2019 passed by District Consumer Disputes Redressal Commission, Jalandhar (now in short, "the District Commission"), wherein the District Commission partly accepted the complaint of the complainant and the following relief has been granted:

"19. In the light of above detailed discussion, the complaint of the complainant is partly accepted and accordingly, OP No.1, 2 and 4, who are jointly and severally liable for causing negligence with the wife of the complainant as well as harassment to the complainant, mentally and physically, therefore, the aforesaid OPs are directed to pay damages to the complainant, to the tune of Rs.8,00,000/- and further the aforesaid OPs are directed to pay litigation expenses of Rs.10,000/-".

These both appeals bearing No.45 of 2019 and No.54 of 2019 have been filed by the appellants for setting aside the impugned order of the District Commission. The facts are taken from First Appeal No.45 of 2019 filed by the appellant/OP No.2 titled as "Dr. Amandeep Singh Vs. Gurdial Singh and others".

2. It would be apposite to mention that hereinafter the parties will be referred, as have been arrayed before the District Commission.

3. Brief facts of the case for disposal of the appeals are that the complainant got admitted his wife in OP No.1 hospital on 08.02.2017, as she was suffering from some ailment. Seeing the seriousness of the ailment, she was shifted to ICU at about 2:00 PM for want of Centre Line Fixation. At the time of admission in the said hospital, it was brought to the notice of the complainant that as the veins of his wife could not be traced out, the OPs were to install Centre lining in her body for the purpose of her treatment. The OPs also assured the complainant that the centre lining is a safe process and the same is one and only route of administration in dire emergency, when patient might go into COMA and a result of such circumstances his wife would get the doses of medicines through that centre lining and only after the installation of the same the treatment could have been possible and not otherwise. The complainant immediately gave his consent for installation of centre lining in her body and also purchased all the material required for the said process and handed over the same to OPs No.2 and 3, who assured the complainant that they would initiate the centre line process on his wife and treatment would be started immediately within no time of its installation. The complainant was under the bonafide belief that the treatment had been initiated on his wife and OP No.2, when contacted, every time assured that the treatment process had been started and condition of his wife was stable and she was responding to the treatment. But on 09.02.2017, when the complainant got to see his wife at about 01:00 PM, he was shocked to see that there was no centre line process on the body of his wife. The complainant wanted to confirm the same from OP No.2, who started avoiding him and kept on putting off the matter and assured that the treatment had been initiated upon his wife. The complainant requested OPs No.2 and 3 to install the centre lining on his wife's body for

administration of the medicines in her body, but they did not bother and pay any heed to the genuine request of the complainant for the reasons best known to them. It was further alleged that after getting the requisite documents in his hand, the complainant came to know that the OPs gave treatment for Type II Diabetic Mellitus, whereas the wife of the complainant was actually suffering from Hypothyroidism and Hypertension, which was very much evident from Complicated Chronic Disease Certificate bearing No.12176 dated 21.09.2015 issued by Guru Nanak Dev Hospital, Amritsar. The wife of the complainant was never a patient of Diabetes and the OPs adopted and gave different line of treatment, which was not required in her case, which also resulted in death of his wife for which the OPs were wholly and solely responsible. It was further alleged that due to wrong line treatment, no glucose was administered in the body of complainant's wife, due to which the condition seemed to be deteriorating. It was pertinent to mention that immediately after admission in ICU, an injection of Doparium was given to his wife to raise her Blood Pressure, despite the fact that she was a declared case of Hypertension, which shows the negligence and wrong line of treatment being given to his wife. Due to non-processing of the Center Line Process and due to non-providing of necessary and requisite treatment, the wife of the complainant succumbed to her ailment and she was declared dead on 10.02.2017 at 06:20 AM. The act and conduct of the OPs amounts to negligence and deficiency in service. Hence, the complainant filed the consumer complaint before the District Commission and sought Rs.18,00,000/- as compensation for causing mental tension, harassment and for loss of life and company due to death of his wife and Rs.33,000/- as litigation expenses.

4. The complaint was contested by the OPs. OPs No.1 to 4 appeared through counsel and filed joint reply. They raised certain preliminary objections as stated in their reply, which are not required to be reproduced here for the sake of brevity. On merits, OPs No.1 to 4 stated that they were rendering the best of medical services in the field at their Super Specialty Hospital. Smt. Surinder Kaur, wife of the complainant, was brought to the hospital in a very serious and critical condition. The patient was having right sided pneumonia, portal vein thrombosis, sepsis acute liver and kidney injury. The patient was admitted in the ICU because of her serious condition and not for the want of a central line insertion. The veins of the patient could be traced out and intravenous cannula was inserted. At the time of admission and even afterwards, the need for the central line insertion was not felt as the veins were traced out and the medicines were being given to the patient intravenously. On the repeated request of the complainant, the central line set was prescribed to the patient to be kept ready. It was assured to the husband of the patient that if need be, central line would be inserted. However, such need never arose, as the veins of the patient were traced out. Even if there would have been any indication for the insertion of the central line, it would not have been possible to insert the central line as the PT (INR) of the patient was 4.7%. This was a contra indication for the insertion of central line. The prognosis of the patient had been intimated to the complainant right from the time of admission, which is clear from the consent signed by the complainant/the husband of the patient. The line of treatment given to the patient was ethical according to the disease pattern and there was no negligence on the part of the OP No.2 and 3. Blood Pressure of the patient had fallen during her stay in the hospital due to septicemia that the patient had, which is a known complication in such disease and so ionotropes and vasopressors were started. The patient remained admitted in ICU until her death. The patient succumbed to her multiple fatal diseases in a little time over 35 hours after admission. The patient died on account of multiple, fatal, serious diseases which

she was suffering from at the time of admission and not on account of any negligence on the part of the OP No.2, who is an expert doctor having sufficient experience. The history of the patient being diabetic was given by her relative. No insulin was given to the patient and the patient was kept on regular monitoring of the blood sugar. It was found during this monitoring that the blood sugar was going low, so rather dextrose based fluids were given to the patient to raise the blood sugar. The death of the patient was due to grave illness that the patient was suffering from as was evident from the patient file and certified by more than two doctors independently, who have examined the patient. The complaint was false and motivated and therefore, the same was liable to be dismissed. After denying the other averments made in the complaint, OPs No. 1 to 4 prayed for dismissal of the complaint.

5. OP No.5 filed its separate reply and contested the complaint by taking preliminary objections, that there was no privity of contract between the complainant and OP No.5. The OP No.5/Oriental Insurance Co. Ltd. was not necessary party in the present complaint and as such, the complaint was liable to be dismissed on this short score alone. It was further alleged that without admitting any liability, it was submitted that no copy of any Insurance Policy had been provided to OP No.5 for having insured any other OP under the Professional Indemnity Policy. However, in case there was any such policy then the same was between the insured and the insurer and subject to the terms and conditions therein and no claim arisen within one year from the end of the respective policy period and thus, the complaint was liable to be dismissed on this short score alone. It was further submitted that no liability qua the answering respondent could be fixed in the complaint proceedings. The matter inter-se the insured and the insurer was to be decided by the insurance company by determining whether any claim was covered under the respective policy obtained by the concerned doctor or hospital and thereafter, the liability of the insurance company, if any, was to be determined by the competent authority of the company, as per the terms and conditions/exclusion clauses of the policy. After denying the other averments made in the complaint, OP No.5 prayed for dismissal of the complaint qua it.

6. The parties led their evidence in support of their respective contentions and the District Commission after going through the record and hearing learned counsel for the parties, partly allowed the complaint of the complainant, vide impugned order as above. Aggrieved with the same appeals have been filed by the appellants.

7. Notice of the appeal was issued to the respondents through registered post. All the respondents, except respondent No.3, appeared through counsel but respondent No.3 did not appear despite service, therefore, she was proceeded against exparte vide order dated 02.05.2019.

8. We have heard the Ld.counsel for the appellant and respondents, except respondent No.3, and have carefully gone through the written submissions filed by them and the record of the case.

9. Learned counsel for the appellant/OPs No.2, has vehemently contended on the similar lines as pleaded in written reply filed before the District Commission. The learned counsel further contended that the appellant specifically stated before the District Commission that if needed central line would be inserted. It was never assured that centre line would definitely be inserted. It

was only for the safety that centre line was called for but at that time the same was not inserted as the vein of the patient was traced out and intravenous cannula was inserted and the medicines were given to the patient intravenously. It was only on the repeated request of respondent No.1/complainant, the central line set was prescribed to the patient to keep it ready. Moreover, the prognosis of his wife was intimated to him right from the time of admission, which can be barely perused from the consent signed by him. The learned counsel further contended that on 08.02.2017 at the first moment i.e. at 1:50 PM Blood Sugar of the patient was checked and diabetes was found 163 mg/dl and further at 2:00 PM the same was 152 mg/dl, which shows that patient was suffering from diabetes. Moreover, the diabetes count stated by the complainant in his complaint was of Urine and as per medical rules and expert, the diabetes count for the Blood Sugar and Urine are different. The learned counsel further contended that with regard to the Blood Pressure of the patient is concerned, it had fallen during her stay in the hospital due to septicemia that the patient had, which is a known complication in such disease and so ionotropes and vasopressors were started. The patient succumbed to her multiple fatal diseases in a little time over 35 hours after admission. The death of the patient was due to grave illness, which is evident from the patient file and certified by more than two doctors independently, who have examined her. Alleging no negligence and deficiency in service on the part of the appellant, the learned counsel for the appellant prayed for acceptance of the appeal and setting aside of the impugned order.

10. Learned counsel for the respondents No.2 & 4/OPs No.1 & 4 (Appellants in FA No.54 of 2019) has also vehemently contended on the similar lines as pleaded in written reply filed before the District Commission. The learned counsel further submits that the impugned order passed by the District Commission is wholly perverse, erroneous and not sustainable in the eyes of law. There is no conclusive evidence on record that the wife of respondent No.1/complainant died on account of any negligence and deficiency in service on the part of appellants. The observations made by the District Commission are based on exemptions and presumptions. There is no opinion nor any report of Medical Board, which was to be constituted by Civil Surgeon, Civil Hospital, Jalandhar regarding the act and conduct of OP No.2 nor any medical expert has been examined by the complainant. The Learned counsel further argued that the patient remained admitted in the said Hospital for barely 35 hours and was brought in a critical condition suffering from multiple fatal diseases. The patient was provided the best possible medical care and remained admitted in Intensive Care Unit under the supervision of expert Medical Practitioner and Para-medical Staff throughout her treatment. The District Commission has wrongly discharged OP No.5, insurance company, from the liability of the payment of compensation awarded to the complainant. Appellant/OP No.2 got himself insured from OP No.5 for professional indemnity and the very purpose of the insurance was that in case any such claim is awarded against OP No.2, the OP No.5 shall pay the same without questioning the merits of such claim. The District Commission has wrongly held that the line of treatment given to the patient was wrong. In fact, the line of treatment given to the patient was ethical according to her disease pattern and there was no negligence on the part of the appellant/OPs. The learned counsel for the respondents No.2 & 4/OPs No.1 & 4 prayed for acceptance of appeal and for setting aside of the impugned order.

11. On the other hand, learned counsel for the respondent No.1/complainant, argued that the order passed by the District Commission is a well reasoned order and had rightly awarded the

compensation on account of negligence and deficiency in service. The learned counsel argued on the similar lines as mentioned in his complaint, which is not required to be reproduced here for the sake of brevity. The Learned counsel prayed for dismissal of appeal.

12. The learned counsel for respondent/OP No.5, insurance company, argued that OP No.5 is only a performa respondent in the present appeals, as no direction has been issued against it. Hence, the learned counsel prayed for dismissal of the appeals qua insurance company/OP No.5

13. We have given thoughtful consideration to the contentions raised by appellant and the appearing respondents.

14. The factual matrix of the case are that the respondent No.1/complainant got admitted his wife on 08.02.2017 in the hospital of appellant/OP No.1 for treatment of her ailment under the supervision of appellant/OP No.2. The wife of the complainant was shifted to ICU of the said hospital on the same day at about 02:00 pm due to her critical condition. Thereafter on 10.02.2017 at about

06.:20 am, she succumbed to her ailment and was declared dead. Respondent No.1/complainant has alleged that due to non processing of central line process and due to adopting wrong line of treatment by the appellants/OPs, his wife succumbed to her ailment. Hence, a consumer complaint was filed by the complainant before the District Commission, which was partly accepted against the OPs No.1,2 & 4, vide impugned order as above. Aggrieved by the said order of the District Commission, two separate appeals have been preferred, one by appellant/OP No.2 i.e. FA No.45 of 2019 and another by appellants/OP No.1 & 4 i.e. FA No.54 of 2019 for setting aside the impugned order.

15. The foremost grievance raised by the appellant/OP No.2 in the present appeal is that the District Commission has failed to appreciate the fact that there is no deficiency in service or negligence on the part of the appellant/OP No.2, who is a professional doctor working with the said hospital. In reply to the allegations levelled by the respondent No.1/complainant that his wife succumbed to her ailment due to non-fixation of Central line, the appellant/OP No.2 has vehemently contended and pleaded that in the present case the need for fixation of Central Line did not arise as the veins of the patient were traceable and medicines were given to the patient intravenously as well as orally. Moreover, it would not have been possible to insert the central line as the PT(INR) report of the patient was 4.7%, which was very high. On the other hand, the contention of respondent No.1/complainant is that his wife was shifted to ICU on 08.07.2017 at about 2:00 pm for want of centre line fixation and it was assured by OPs No.2 & 3 that they would initiate the centre line process on the patient and treatment would be started immediately. But they failed to do so even after the centre line kit was handed over to them.

16. Now, the foremost point for consideration before us is whether the District Commission has rightly held the appellant negligent for not inserting the central line for administration of the medicine to the patient or not? To determine this point, we have carefully perused the entire evidence on record. From the perusal of In-Patient Record, Ex. OP-1, it shows that the wife of the complainant was admitted at Punjab Institute of Medical Science, OP-1-hospital, on 08.02.2017 in a

critical condition, which can be assessed as under the heads Provisional Diagnosis and Final Diagnosis, it is stated as under:-

"Provisional Diagnosis: Type-II DM, Hypothyroidism, UTI, LRTI, Septicemia, ALI(Hepatitis), AKI(CAAKI), Hepatic Encephalopathy, CAD, Cholelithiasis, Morbid obesity and Portal Vein Thrombosis.

Final Diagnosis: Type-II DM, Hypothyroidism, UTI, LRTI, Septicemia, ALI(Hepatitis), AKI(CAAKI), Hepatic Encephalopathy, CAD, Cholelithiasis and Morbid Obesity".

The history and diagnosis of the patient's illness clearly show that she was brought to the hospital in a very serious and critical condition as she was already a patient of Type-II DM, Hypothyroidism, suffering from UTI infection, right side pneumonia, septicemia, acute liver and kidney injury, CAD, Cholelithiasis and Morbid obesity. The doctor always takes the history of the patient from either the patient or his relatives to arrive at the provisional and final diagnosis and accordingly begin with the line of treatment. It is not the case of the respondent No.1/complainant that the OPs did not follow the standard procedure that was required to be given to such a patient at the relevant time. The patient was admitted in ICU because of the seriousness of the condition of the patient and not for want of central line insertion. The appellant/OPs have placed on record Intake- output Chart, Ex. OP-8. A perusal of the same shows that on 08.02.2017 fluids were given to the patient through Intravenous Cannula inserted into her vein and total intake was 1200 ml and output of urine was 400 ml. Similarly, on 09.02.2017 fluids were given through vein as well as orally and it also shows that there was total intake of 2000 ml and output of urine was 500 ml. Ex. OP-10 and OP- 11 are Drug Administration Record, which show that doses of medicine were administered frequently to the patient through IV route on 08.02.2017, 09.02.2017 and even on 10.02.2017 i.e. on the date of her death. The said documents placed on record by the appellant/OPs, corroborate his version that the need for central line insertion was not felt as the veins were present and the medicines were being given to the patient intravenously. Ex. OP-16 is Blood test report conducted by OPs Hospital, which shows that PT (INR) of the patient was 4.7%. The said ratio was very high, which means that the patient had an increased risk of bleeding as the blood was taking longer to clot. It also supports the version of the appellant that due to higher ratio of PT (INR) of the patient, it was not possible to insert the central line in her body. The contention of respondent No.1/complainant is that his wife was admitted in the hospital on 08.02.2017 for want of centre line fixation and in support of his contention he placed on record Ex.C-15 i.e. requisition slip wherein centre line kit was prescribed by the treating doctor. A perusal of the said slip shows that the same was issued on 09.02.2017 i.e. after the date of admission on 08.02.2017, which also proves the version of the appellant that the central line set was prescribed to the patient to be kept ready and same would be inserted, if needed. But no need arose as the veins of the patient were traced out. Accordingly, we find no force in the contention of respondent No.1/complainant that OPs are negligent in not inserting the center line in the body of his wife. Rather, we agree in the affirmative with the contention of learned counsel for the appellant/OP No.2 that there was no need of center line insertion in the body of the patient as the veins were traced out. The District Commission has wrongly held the OPs negligent for not inserting the central line for administration of the medicine to the patient Surinder Kaur.



17. Another plea of the appellant is that the District Commission, while relying on the allegation levelled by respondent No.1/complainant that the line of treatment given to the patient was wrong, held the OPs negligent, whereas, the line of treatment given to the patient was ethical according to her disease pattern and there was no negligence on the part of the appellant/OPs. On the other hand, the contention of respondent No.1/complainant is that his wife was never a patient of Type-II DM and the OPs adopted and gave different line of treatment, which was not required in her case. In support of his contention respondent No.1/complainant placed on record Ex. C-8 i.e. Blood Test report issued by 'Paramjit Clinical Laboratory & Microbiology Culture Lab', wherein Blood Sugar(Fasting) of his wife was 84 mg/dl on 08.02.2017 i.e on the date of admission of patient in the hospital of OP No.1. The complainant further placed on record Ex. C-23 i.e. Urine Analysis report issued by 'FA Speciality Lab' on dated 18.01.2017, wherein Sugar level of patient Surinder Kaur was mentioned as NIL. Similarly, in Chemical Examination report issued by 'Kripa Clinical Laboratory', Ex. C-24, her urine analysis for Sugar was shown as NIL on 06.02.2017. On the other hand, the appellant/OP No.2 relied on certain documents i.e In Patient Record, Ex. OP-1, wherein the wife of the complainant was shown a patient of Type II DM under the heading of Provisional Diagnosis as well as Final Diagnosis. It is a matter of fact that either the patient or his close relative would narrate the history of patient to the doctor, on the basis of which, the doctor arrived at mentioning provisional diagnosis as Type II DM. Ex. OP-2 i.e. Fatal Document-Check list also shows that patient Surinder Kaur was Diagnosed for Type II DM as well as other multiple diseases i.e. Hypothyroidism, UTI, LRTI, ALI(Hepatitis), AKI(CAAKI), Hepatic Encephalopathy, CAD, Gallbladder Stone, Morbid obesity and the said check list was duly certified by the Consultant Doctor and Head of Department of OP Hospital. Ex. OP- 3, is the Medical Certificate of cause of death, wherein the immediate cause of death is Cardiopulmonary Arrest and Antecedent cause of death is stated to be Type II DM and Hypothyroidism, AKI, ALI, CAD and Hepatic Encephalopathy. We have also perused Ex. OP-7 i.e. Blood Sugar/Insulin Chart prepared by the Staff Member of OP Hospital, wherein blood sugar was checked every 4 hours and the level of Sugar was observed as under:-

| Date       | Time     | RBS       |
|------------|----------|-----------|
| 08.02.2017 | 1:50 pm  | 163 mg/dl |
| 08.02.2017 | 10:00 pm | 108 mg/dl |
|            | 02:00 am | 152 mg/dl |
| 09.02.2017 | 06:00 am | 73 mg/dl  |
|            | 10:00 am | 76 mg/dl  |
|            | 02:00 pm | 78 mg/dl  |
|            | 06:00 pm | 41 mg/dl  |
| 09.02.2017 | 07:00 pm | 102 mg/dl |
| 09.02.2017 | 11:00 pm | 67 mg/dl  |
| 09.02.2017 | 03:00 am | 126 mg/dl |
| 10.02.2017 | 06:00 am | 253 mg/dl |

From the perusal of above said chart, we observe that the blood sugar level of patient Surinder Kaur was frequently going up and down during the period of stay of the patient in the OP No.1 Hospital. On the date of admission 08.02.2017, the Blood Sugar level was '163 mg/dl' and on 09.02.2017, it was around normal range but on 10.02.2017 it again went upto '253 mg/dl'. Accordingly, from the

above said documents relied upon by the appellant/OP No.2, we are of the view that the patient was suffering from Type II DM as is also evident from the blood sugar/insulin chart( Ex.OP-7) prepared by the OP Hospital. Hence, there is force in the contention of appellant/OP No.2 that the patient was diabetic and the treatment given to the patient was ethical as per the standard medical protocol and according to her disease pattern. The District Commission has wrongly relied on the version of the respondent No.1/complainant that wife of the complainant was never a patient of Diabetes and the OPs adopted and gave different line treatment, which was not required in her case.

18. As far as the contention of learned counsel for the appellant/OP No.2 that despite best efforts to cure the patient, she died due to her grave illness and not by negligence on the part of the appellant is concerned, it has been proved on record that the wife of the complainant, namely; Surinder Kaur was brought to the hospital in a very serious and critical condition. At the time of admission, she was suffering from various fatal diseases as is evident from Admission Request Form Ex. OP-21 and Ex.OP-1 and OP-2, as discussed above. Further the progress note Ex. OP-24, also shows that the patient was diagnosed with Type II DM, Hypothyroidism, UTI, LRTI, Septicemia, ALI(Hepatitis), AKI (CAAKI), Hepatic Encephalopathy, CAD and Cholelithiasis and was shifted to ICU due to her critical condition. Even, the blood test reports, Ex. OP-12 to OP-19, produced by the appellant/OPs prove that patient was suffering from multiple fatal diseases. The blood pressure of the patient had fallen during her stay in the hospital due to Septicemia that the patient had, which is a known complication in such cases and so ionotropes and vasopressors were started to raise the blood pressure. The prognosis of the patient had been intimated to respondent No.1/complainant right from the time of admission, which is clear from the Informed Consent for Admission form, Ex. OP-22, duly signed by him as the patient was brought to hospital in a critical state. By signing the said consent letter the respondent No.1/complainant gave his consent to the terms of the said letter. The patient died on account of multiple, fatal, serious diseases , which she was suffering from at the time of admission and not on account of any negligence on the part of the appellant/OP No.2, who is an expert doctor having sufficient experience. Hence, we are of the opinion that there is force in the contention of appellant/OP No.2 that despite best efforts, the wife of complainant died due to her grave illness and there was no deficiency/negligence on the part of the appellant/OP No.2.

19. Another ground of appeal raised by the appellant/OP No.2 is that the District Commission has based its observation on assumption and presumption. It has been submitted that neither any expert opinion nor any report of medical board, which was to be constituted by Civil Surgeon, Civil Hospital, Jalandhar regarding the act and conduct of appellant/OP No.2, has been examined. In this regard, we are of the view that since the allegation of the respondent No.1/complainant is of medical negligence on the part of OPs, especially in a case where the patient was brought in a critical condition in the hospital, the ends of justice would have squarely been met and a medical board or opinion of medical expert would have clinched the controversy had the District Commission summoned for the same. Although it is not imperative to constitute a medical board but sometimes, the report of an expert would have thrown some light on the issue whether standard medical protocol has been followed or not. The District Commission has also not gone into the depth of the evidence so adduced before it and simply held the Doctor to be negligent on account of two allegations i.e. non fixation of central line and wrong line treatment as alleged by the respondent No.1/complainant. Therefore, we support in affirmative with the contention of the appellant/OP

No.2 that the evidence or medical report is not read into properly by the District Commission.

20. We are fortified with the Judgment passed by the Hon'ble National Commission in the case titled as V.P. Singh Vs. Dr. M.P. Chandrakar & others(NCDRC) 2016(3) C.P.R. 23, wherein it has held that:-

"Consumer Protection Act, 1986 Section 21 Revision Petition- Medical Negligence - Proof - On the basis of laboratory reports, medical records, and the medical board's opinion, it is found that, patient was suffering from advanced liver cirrhosis, and he was in liver failure, thus it was a fatal condition - There is nothing on record to prove that doctor made a very unpleasant remark and advised for liver transplant - OP Hospital treated the patient as per standard of practice - No deviation found from the duty of care - No cure is no negligence - Hospital treated the patient of liver failure with the best possible available mode - No lapses or shortcomings found in the treatment given by OP - Appeal dismissed".

In an another judgment titled as Dr. K.K. Sharma Vs. Fortis Hospital (NCDRC) 2010(1) C.P.J. 271, the Hon'ble National Commission has held as under:-

"Consumer Protection Act, 1986 Sections 2(1)(g) and 19 Appeal - Medical negligence - Proof - Injuries due to accident - Medical record shows that Patient at the time of admission into Hospital was in a coma, was in a case of progressive worsening of the sensorium on account of head injury and infection, fractured in both bones in the left leg, was hypertensive, diabetic, had bronchial asthma and bed sores on the left thigh and back - He was a high risk Patient considering his age of 86 years and being in condition stated, at the time of admission into Hospital - No material on record to establish that treatment given to Patient was against norms of normal standard medical parlance - No evidence on record to establish that the Appellant Hospital/ doctors/ staff have been negligent in the treatment of the patient - All efforts were made in accordance with standards of normal medical parlance - No medical negligence can be attributed to Respondents- Appeal dismissed".

Further the Hon'ble Supreme Court in its judgment titled as Kusum Sharma & others Vs. Batra Hospital & Medical Research Centre & others (JT 2010 (2) SC 7), while dealing with the cases of negligence, examined various pronouncements and while deciding whether the medical profession is guilty of medical negligence held that following well known principles must be kept in view:-

"I. Negligence is the breach of a duty exercised by omission to do something which a reasonable man, guided by those considerations which ordinarily regulate the conduct of human affairs, would do, or doing something which a prudent and reasonable man would not do.

II. Negligence is an essential ingredient of the offence. The negligence to be established by the prosecution must be culpable or gross and not the negligence

merely based upon an error of judgment.

III. The medical professional is expected to bring a reasonable degree of skill and knowledge and must exercise a reasonable degree of care. Neither the very highest nor a very low degree of care and competence judged in the light of the particular circumstances of each case is what the law requires.

IV. A medical professional would be liable only where his conduct fell below that of the standards of a reasonably competent practitioner in his field.

V. In the realm of diagnosis and treatment there is scope for genuine difference of opinion and one professional doctor is clearly not negligent merely because his conclusion differs from that of other professional doctor. VI. The medical professional is often called upon to adopt a procedure which involves higher element of risk, but which he honestly believes as providing greater chances of success for the patient rather than a procedure involving lesser risk but higher chances of failure. Just because a professional looking to the gravity of illness has taken higher element of risk redeem the patient out of his/her suffering which did not yield the desired result may not amount to negligence.

VII. Negligence cannot be attributed to a doctor so long as he performs his duties with reasonable skill and competence. Merely because the doctor chooses one course of action in preference to the other one available, he would not be liable if the course of action chosen by him was acceptable to the medical profession. VIII. It would not be conducive to the efficiency of the medical profession if no Doctor could administer medicine without a halter round his neck.

IX. It is our bounden duty and obligation of the Civil society to ensure that the medical professionals are not unnecessary harassed or humiliated so that they can perform their professional duties without fear and apprehension.

X. The medical practitioners at times also have to be saved from such a class of complainants who use criminal process as a tool for pressurizing the medical professionals/hospitals particularly private hospitals or clinics for extracting uncalled for compensation. Such malicious proceedings deserve to be discarded against the medical practitioners.

XI. The medical professionals are entitled to get protection so long as they perform their duties with reasonable skill and competence and in the interest of the patients. The interest and welfare of the patients have to be paramount for the medical professionals.

21. Sequel to our above discussion and in view of ratio of above referred judgments, we come to the conclusion that the wife of the complainant was a high risk patient considering her age of 61 years

and being in condition as stated, at the time of admission into the hospital. There is no material on record to establish that treatment given to patient was against norms of normal standard medical protocol. Therefore, we are of the considered view that the negligence cannot be attributed to appellant/OP No.2 so long as he performed his duties with reasonable skill and competence and has given treatment as per norms. Accordingly, we allow the appeal of the appellant/OP No.2 and the impugned order of the District Commission is hereby set aside.

22. The appellant had deposited a sum of Rs.25,000/- at the time of filing of the appeal. This amount alongwith interest, which has accrued thereon, if any, shall be remitted by the registry by way of crossed cheque/DD to the appellant/OP No.2 after the expiry of limitation period in accordance with law.

23. In view of the reasons and discussion held in First Appeal No.45 of 2019 titled as "Dr. Amandeep Singh Vs. Gurdial Singh & others", this appeal No.54 of 2019 titled as 'Punjab Institute of Medical Science & another Vs. Gurdial Singh and other', is also allowed and the impugned order qua appellants is also set aside as there was no negligence or deficiency in service on the part of the appellants/OP No.1 & 4.

24. The appellants had deposited a sum of Rs.25,000/- at the time of filing of the appeal. They further deposited an amount of Rs.5,60,000/- in compliance with the order dated 18.02.2019. These amount alongwith interest, which has accrued thereon, if any, shall be remitted by the registry by way of crossed cheque/DD to the appellants/OP No.1 & 4 after the expiry of limitation period in accordance with law.

25. The appeals could not be decided within the stipulated period due to heavy pendency of Court cases and non-sitting of this Commission due to pandemic of Covid-19.

(H.P.S. Mahal) PRESIDING JUDICIAL MEMBER (KIRAN SIBAL) MEMBER May 25th, 2022 (dv)